

The 2026 Healthcare and DME Public-Sector Opportunity Report

A practical guide to VA procurement, Medicare supplier signals, funded providers, recipients, channel partners, and source-backed next actions.

6 lanes Procurement, reimbursement, recipients, channels, grants, and policy	2026 Current CMS enrollment constraint clearly separated from sales paths	30 days A focused operating plan for qualifying the first target list
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Built for founders and revenue teams that need credible targets, clear revenue motions, contact paths, and next actions - not a generic list of grants or contracts.

Opportunity Scanner by Opportunity Systems

Research current as of July 11, 2026. Always recheck time-sensitive official sources before acting.

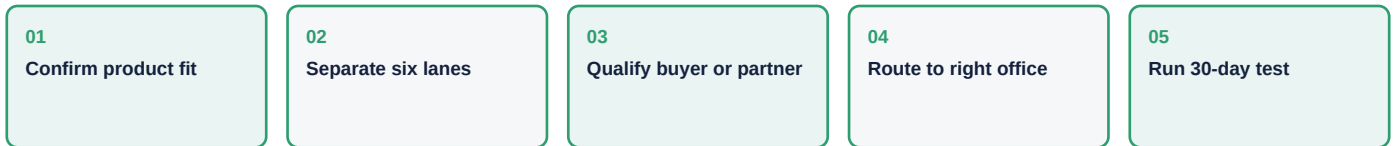
The opportunity landscape

Use these lanes to separate records that imply different buyers, partners, timing, and next actions.

Six evidence lanes to investigate



From signal to an owned revenue action



These diagrams are operating maps, not a promise that every lane is open. The report explains the evidence, constraints, and qualification checks behind each route.

Offer and audience

This report is for healthcare suppliers, DMEPOS companies, medical-supply manufacturers and distributors, rehabilitation and mobility businesses, healthcare technology companies, and revenue teams evaluating public-sector growth.

Conversion promise: Understand the distinct public-sector opportunity paths in healthcare and use a repeatable method to qualify them. The report does not determine Medicare eligibility, accreditation, coding, reimbursement, contracting eligibility, clinical compliance, or revenue potential.

Executive brief

Healthcare public-sector opportunity is not one market. It includes agency procurement, Federal Supply Schedule routes, Medicare supplier and reimbursement requirements, publicly funded health centers and providers, grant recipients, distributors, prime vendors, and policy signals. Each lane uses different evidence and demands a different action.

National health spending provides context for the scale and policy importance of healthcare, but it is not the addressable market for a DME or medical-supply company. CMS projects national health expenditure growth and publishes Medicare data, fee schedules, supplier guidance, and policy changes. VA publishes medical equipment and supply schedule information. HRSA publishes current and historical grant and health-center data. SAM.gov and USAspending provide procurement and federal award evidence.

The highest-value work is separating these lanes before outreach. A reimbursement rule may require Monitor Policy or supplier-readiness work. A VA schedule may support a contracting pathway. A HRSA recipient may support funded-buyer or partner research. A prior award may reveal an incumbent, distributor, or agency pattern without proving current demand.

The correct output is a focused action table in which every row identifies offer fit, official evidence and period, target, route, constraint, and next validation task. A compliance requirement can be strategically important without being a sales lead.

State of the market and opportunity signals

Trend 1: Healthcare spending and public funding remain large, but broad totals require restraint

CMS's current National Health Expenditure fact sheet projects average NHE growth of 5.4 percent from 2025 through 2034, with Medicare spending projected to grow 7.7 percent per year on average. HRSA reports more than 5,500 grant awards annually and reports over \$11.1 billion in FY2025 grants across primary care, health workforce, HIV/AIDS, maternal and child health, rural health, and other programs.

Business implication: Public money affects many healthcare organizations and delivery systems, but those totals combine categories that are not interchangeable and do not measure DME demand.

Recommended action: Use program, recipient, service, product, geography, and time filters to find the narrow records that map to the offer.

Trend 2: Medicare supplier participation is a readiness and compliance lane, not a generic sales lead

CMS states that DMEPOS suppliers seeking Medicare reimbursement generally must obtain accreditation from a CMS-approved organization, enroll in Medicare, and post a \$50,000 surety bond for each NPI they maintain, subject to stated exemptions and program details.

Business implication: A reimbursement or supplier record may identify market structure, competitors, referring patterns, or compliance work. It does not automatically create a buyer or open opportunity.

Recommended action: Route Medicare signals to supplier-readiness, reimbursement, compliance, product-category, or provider-market research before deciding whether outreach is appropriate.

Trend 3: A temporary 2026 enrollment moratorium changes the direct supplier path

CMS reports that on February 27, 2026 it implemented a temporary nationwide moratorium on initial Medicare enrollment for specified DMEPOS medical supply company categories. CMS states the moratorium lasts six months and may be extended in six-month increments.

Business implication: A company affected by the moratorium should not interpret supplier-enrollment content as an immediately available path. The status is time-sensitive and does not eliminate separate procurement, distribution, manufacturer, technology, service, funded-provider, or partner opportunities.

Recommended action: Verify the current CMS moratorium page and definitions before taking enrollment action. Consult the appropriate enrollment contractor or qualified advisor for company-specific questions.

Trend 4: VA procurement includes category-specific schedule routes

VA's Schedule 65 II A covers medical equipment and supplies across categories such as surgical dressings, instruments, hospital equipment, clothing, ophthalmic equipment, kits, and medical equipment maintenance. VA identifies SAM.gov as the official source for government contract opportunities while publishing schedule solicitation materials and ordering information.

Business implication: Product category, Federal Supply Code, NAICS, schedule requirements, and ordering route matter. A company may also participate through a distributor, schedule holder, prime, or subcontracting relationship.

Recommended action: Map products to the applicable VA schedule and codes, inspect active notices separately, and compare direct schedule, agency procurement, distributor, and partner motions.

Trend 5: Funded providers and recipients create a distinct buyer and partner lane

HRSA publishes active and awarded grants, health-center awardee data, and service-delivery sites. HRSA notes that FY2026 award data is preliminary and current-year-to-date, and defines an active grant by project-period end date rather than proof of new current-year funding.

Business implication: A funded health center or recipient may be a buyer, partner, implementation site, or research target, but the funding purpose and active project scope must fit the offer.

Recommended action: Verify program purpose, project period, recipient role, and likely purchasing authority before creating outreach.

Trend 6: Round 2028 competitive bidding is a readiness signal, not yet a contract award

CMS states that the program is in a temporary gap after Round 2021 contracts expired on December 31, 2023. Round 2028 covers Class II continuous glucose monitors and insulin pumps, urological supplies, ostomy supplies, hydrophilic urinary catheters, and off-the-shelf back, knee, and upper-extremity braces. Published registration and bidding dates remain targets; CMS says the round will begin no later than January 1, 2028.

Business implication: Affected suppliers have a readiness lane around enrollment records, licensing, category/location accreditation, and partner strategy. The announcement does not mean registration is open or a contract is available.

Recommended action: Assign an owner to monitor the official CMS and Competitive Bidding Implementation Contractor pages. Inventory category and location accreditation now, but verify actual registration and bid-window dates before representing the opportunity as active.

Six healthcare opportunity lanes

Lane	Evidence	Typical revenue motion	First validation
VA and agency procurement	Schedule pages, SAM.gov notices, award history, vendor portals	Sell to Agency; Channel / Distributor or Motion	Product code, notice status, contract vehicle, buyer route
Medicare supplier and reimbursement	CMS enrollment, accreditation, fee schedule, policy, claims data	Monitor Policy; Research Only; supplier-readiness	Current rule, eligibility, code, geography, data period

Lane	Evidence	Typical revenue motion	First validation
Funded providers and health centers	HRSA grants, USAspending, recipient and site data	Sell to Funded Buyer; Partner with Recipient	Program purpose, project period, purchasing role
Prime, distributor, and manufacturer channels	Award records, VA catalogs, SBA subcontracting, recipient sites	Sell to Award Recipient; Channel / Distributor or Motion	Existing vehicle, category coverage, partner fit
Direct grants and assistance	Grants.gov and agency program notices	Direct Apply; Partner with Recipient	Eligibility, status, deadline, allowable use
Policy and emerging demand	Federal Register, CMS rules, agency guidance	Monitor Policy; Research Only	Effective date, final versus proposed rule, business impact

Healthcare source and action map

Healthcare research becomes safer when each source is assigned a narrow job.

Source record	Verify	Contact path	Do not infer
CMS enrollment/accreditation	Applicable supplier type, NPI, location, category, exemption	NPE contractor, approved accreditor, qualified advisor	Compliance guidance is a buyer lead
CMS fee schedule/public-use data	Year, geography, HCPCS, population, suppression, payment definition	Reimbursement lead or payer-policy route	Claims or allowed amounts equal current demand
CMS competitive bidding	Final versus target date, covered category/location, enrollment record	CBIC education/help; NPE contractor	Preparation guidance is an open bid
VA FSS schedule/catalog	Exact item, category, holder, terms, purchasing priority	FSS help, holder, distributor, contracting office	Schedule coverage is an active order
SAM.gov notice/award	Type, amendment, due date, codes, set-aside, vehicle	Notice instructions and contracting contact	Award or sources sought is a bid
HRSA grant/health center	Purpose, project period, operator, purchasing role	Recipient procurement or program operations	Grant amount is available for the product
Grants.gov opportunity	Posted status, eligibility, deadline, cost share	Listed grants contact	Topic fit establishes eligibility

The table deliberately separates buyer-facing routes from readiness routes. A company can have a high-priority compliance task and no supported outreach target; that result is more useful than forcing the record into a sales sequence.

The healthcare opportunity workflow

Step 1: Build a product and readiness profile

Record products, HCPCS or product categories when known, clinical and non-clinical use cases, customers, delivery geography, licenses, certifications, accreditation, reimbursement status, manufacturing or distributor relationships, and implementation capacity. Unknown fields should become validation tasks, not assumptions.

Use one row per offer. Capture exact item and setting; verified HCPCS, FSC, NAICS, or schedule category; commercial role; geography and service model; enrollment, accreditation, licensing, and vehicle readiness; and unresolved evidence gaps. Do not assign an HCPCS code from marketing language alone.

Step 2: Translate into public-sector lanes

Map each offer to the six lanes above. A mobility product may map to VA patient mobility schedules, rehabilitation programs, DMEPOS policy, funded providers, and distributor partners. A healthcare software product may map to health-center recipients, agency modernization, prime implementers, and program operations without being a DMEPOS supplier.

Select no more than two primary lanes for the first 30 days. Ask what event would create revenue: an agency order, a provider purchase, a partnership, or compliant furnishing and reimbursement. If the event cannot be stated plainly, the lane is exploratory.

Step 3: Search the source that matches the question

Use VA schedule and procurement pages for category and vehicle research; SAM.gov for active federal notices; USAspending for award and recipient patterns; CMS for reimbursement, supplier, fee schedule, and claims context; HRSA for recipients, active grants, and health-center sites; Grants.gov for open assistance; Federal Register for policy changes.

Preserve each record's native unit and period. Claims, services, suppliers, allowed amounts, obligations, outlays, awards, and sites are not interchangeable and should not be added into an opportunity-size estimate.

Step 4: Verify status and constraint

Check notice type, deadline, amendment, project period, data year, proposed versus final rule, enrollment restriction, and the definition behind every metric. Recheck the 2026 DMEPOS moratorium immediately before publishing or advising action.

For the moratorium, verify whether the action is an initial enrollment or non-exempt change in majority ownership and whether a listed company type applies. For Round 2028, distinguish preparation guidance from an announced registration or bid window.

Step 5: Choose target and revenue motion

Name the agency, provider, recipient, prime, distributor, manufacturer, program office, or policy owner. Then select one motion. Do not turn a CMS data record into a sales lead unless a separate buyer path exists.

Separate the **clinical user**, **economic buyer**, **procurement route**, and **compliance owner**. Outreach follows the economic and access roles; discovery incorporates the clinical user.

Step 6: Build a compliant contact path and next action

Prefer procurement contacts, schedule support, program offices, official vendor registration, recipient leadership, prime or distributor business-development roles, and documented source contacts. The next action should validate category fit, active need, purchasing authority, or partner role.

Match the opening to the lane. Follow notice instructions for agency procurement; give a VA holder or distributor exact category fit and delivery capability; ask a funded provider how related purchasing is handled without implying grant money is available; and route Medicare readiness questions to the official contractor, accreditor, or qualified advisor. Source contacts may be intended for procedural questions, not promotion.

Healthcare qualification gates

Score fit High, Medium, or Low, then apply these gates before a row reaches workflow:

Gate	Pass condition	Downgrade or reject when
Product gate	Exact product/service and use case match the record	Match depends on generic terms such as medical, equipment, or rural
Period gate	Notice, rule, award, project, or dataset period is recorded and useful	Deadline passed, data lag is material, or status is unknown
Authority gate	Buyer, recipient, contract holder, or compliance owner can be named	Only a clinical user or unrelated directory entry is known
Route gate	A bid, order, billing, distributor, partner, or monitoring path is supported	The record shows context but no legitimate action route

Gate	Pass condition	Downgrade or reject when
Readiness gate	Known enrollment, accreditation, licensing, vehicle, geography, and capacity constraints are manageable	A hard requirement is unmet or not yet understood
Evidence gate	Official source supports the plain-language claim and caveat	The interpretation exceeds the source definition

Mark **Pursue**, **Validate**, **Monitor**, or **Reject**. Record why: wrong product/entity, expired, ineligible, geography, vehicle or compliance gap, no purchasing path, duplicate, or stale data. Every monitor row needs a trigger and recheck date.

Worked illustrative example

Company context: A manufacturer sells rehabilitation supports and related clinical supplies through distributors.

Evidence: VA Schedule 65 II A includes relevant medical and surgical supply categories; an historical federal award shows an incumbent supplier serving a related category; a HRSA-funded provider operates rehabilitation services in the target region.

Interpretation: The schedule is a contracting route, the award is market evidence, and the funded provider is a possible buyer or partner. None proves an active purchase.

Primary motion: Channel / Distributor Motion through a qualified schedule holder or incumbent supplier.

Secondary research: Validate whether the funded provider buys the category and whether a separate local procurement route exists.

Next action: Map exact product codes and schedule requirements, then identify the distributor or contract owner responsible for the category. This is illustrative, not a customer result.

30-day action plan

Period	Work	Deliverable
Days 1-5	Build offer-level readiness rows; verify codes and gaps; choose two lanes	Readiness inventory
Days 6-10	Check moratorium, VA route, Round 2028 relevance, and official sources	Constraint memo and 15-25 authenticated records
Days 11-17	Apply gates and rejection codes; shortlist up to 10 signals	Five rows selected for validation
Days 18-24	Map user, buyer, access route, compliance owner, and contact path; resolve the largest unknown	Decision-ready action rows
Days 25-30	Prepare approved outreach or readiness tasks; schedule status rechecks; review lessons	Owned actions and next-month scan brief

Track exact offer match, official source, period, target, route, constraint, next action, owner, and recheck date. Commercial outcomes include qualified conversation, partner review, bid/no-bid decision, readiness work, or disqualification.

Limits and warnings

- This report is not legal, clinical, coding, accreditation, reimbursement, procurement, or eligibility advice.
- CMS policies, moratoria, fee schedules, and enrollment requirements can change. Verify current official guidance.
- National health spending and HRSA totals are context, not DME market size or sales potential.
- Medicare claims datasets can exclude Medicare Advantage and other populations, suppress small counts, and lag the current year.
- A supplier directory entry is not proof of active operations, purchasing intent, or product fit.
- A grant recipient is not automatically a buyer.
- VA schedule coverage does not guarantee an award or order.

- Historical awards reveal patterns but are not open opportunities.

Next-action checklist

- Define the product category and use case.
- Confirm whether Medicare supplier rules apply to the company and product.
- Verify the current enrollment moratorium status.
- Map relevant VA schedule, FSC, NAICS, and active notice routes.
- Identify funded providers and recipients whose program purpose fits.
- Compare direct, distributor, prime, and partner motions.
- Preserve source dates and data-period limitations.
- Assign a target, contact path, and one validation task.
- Screen out records that rely on broad healthcare keywords only.
- Recheck live official sources before acting.
- Confirm the dataset or award period before comparing organizations.
- Give every monitor item a trigger and recheck date.

Evidence and source ledger

Claim or use	Official source	Retrieval date	Caveat
Current NHE projections and sponsor context	CMS, official source	2026-07-10	Context only; not DME market size.
DMEPOS enrollment, accreditation, and surety bond steps	CMS, official source	2026-07-10	Requirements and exemptions vary; verify with CMS/enrollment contractor.
Temporary nationwide DMEPOS medical-supply enrollment moratorium	CMS, official source	2026-07-10	Began 2026-02-27; time-limited and extendable; verify current status.
DMEPOS fee schedule files	CMS, official source	2026-07-10	Payment schedule, not demand or buyer intent.
DME supplier and service dataset	CMS Data, official source	2026-07-10	Latest listed data is 2023; Original Medicare FFS scope and suppression apply.
Medical equipment supplier directory	CMS Provider Data Catalog, official source	2026-07-10	Directory entries are not opportunity or market-size proof.
VA Medical Equipment and Supplies Schedule 65 II A	VA OPAL, official source	2026-07-10	Schedule route and categories; inspect current solicitation and SAM.gov notices.
VA schedule ordering context	VA OPAL, official source	2026-07-10	Ordering rules do not prove a current order.
HRSA active and awarded grant datasets	HRSA, official source	2026-07-10	FY2026 is preliminary; active grant definition uses project end date.
HRSA FY2025 grants dashboard	HRSA, official source	2026-07-10	Program total, not product demand or company opportunity.
HRSA health center and site data	HRSA, official source	2026-07-10	Funded-provider context; validate purchasing role and program fit.
Round 2028 DMEPOS competitive-bidding categories and target schedule	CMS, official source	2026-07-11	CMS labels dates as targets; verify registration and bid-window status before acting.
Competitive-bidding temporary gap and Round 2028 status	CMS/CBIC, official source	2026-07-11	Program-status page, not a contract award or proof that bidding is open.
Competitive-bidding accreditation and licensing readiness	CMS, official source	2026-07-11	Requirements are location and product-category sensitive; verify current guidance.
VA FSS product and contract research tool	VA OPAL, official source	2026-07-11	Catalog and contract evidence do not prove current agency demand.

Put this guide to work

Opportunity Scanner reads your company website, finds source-backed public-sector signals, and translates them into target organizations, revenue motions, contact paths, and next actions.

Run a free company scan at opportunityscanner.ai

Recheck official sources before any outreach, application, registration, or bid decision.